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Name: هند السيد مسعود محمد National ID: 27707110104004	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Plus Goserline
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM 21 / 9 / 2020 G

Name	هند السيد مسعود	MRN	٢٢٩٧ / ٢٥
Age	٤٨ سنة	ID	01148777285
Menopausal status	Pie.	Phone	01123663361
NRS SCORE		Marital status	٢٧-٧١١٠١٠٤١٤
BMI		Family history	
CO morbidity	-ve	REASON OF MDT	

Clinical Data

BX:	IDC G13	R	MBC to Liver 1
ER:	8/8	PR:	3/8
HER 2:	+3	Ki67:	40%
Sonomammography:			
7/2025 : Sm : mass 4x3 cm UOQ (R) + Single LN.		TNM Ps 1	
Mets Workup:			
8/2025 : CT : Suspected H.F.L			
Suspected bone.			
9/2025 : B.S : Free			

Recommendations:
9/2025 : Inphasic : bilobar hepatic metastatic Focal Lesion.

Handwritten signature/initials

For Herceptin IV + taxol week

Handwritten notes: Doneo Gussib, ٢٢٩٧/٢٥

MDT OUTCOME & PLAN

Anti taxol week 1 + Herceptin IV

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Handwritten signature		Handwritten signature			

Handwritten notes: ٢٢٩٧/٢٥

Name: هند السيد مسعود محمد		
MR. : 6611		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 49-year-old individual with a clinical history of left breast cancer metastatic to the liver (cT2 N1 M1). The disease was initially characterized as invasive ductal carcinoma, grade 3, with estrogen receptor (ER) 5/8, progesterone receptor (PR) 3/8, and HER2 3+. Prior imaging demonstrated right hepatic focal lesions (segment VI11 and segment V11) measuring approximately 32 × 25 mm, left hepatic focal lesions (segment 2,4) measuring 5.7 mm, and several small vertebral sclerotic lesions pending bone-scan correlation. The patient completed 38 weeks of taxane-based chemotherapy (taxol) together with 13 cycles of trastuzumab (Herceptin). The most recent CT assessment showed stable disease of the hepatic focal lesions with persistence of the previously noted right-lobe lesions, the larger measuring about 32 × 25 mm, and ongoing sclerotic bony lesions. Performance status is 1, and the patient tolerates taxol with only grade 1 peripheral neuropathy. Biopsy pathology confirmed invasive ductal carcinoma, grade 3, with ER 5/8, PR 3/8, and HER2 3+. Imaging findings include the hepatic focal lesions described above and multiple small vertebral sclerotic lesions; bone scan was performed for correlation. Metastatic work-up confirms hepatic and osseous involvement consistent with stage IV disease. The MDT recommended continuation of taxol in combination with intravenous trastuzumab, addition of zoledex and femara for two years, followed by hormonal monitoring (FSH, LH, E2) with maintenance of femara alone. Evaluation will be performed every four months.		
تحريراً في ,		
	الوقت :	التاريخ : 23-09-2026
		الطبيب المعالج :
يعتمد ,		



Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m1137

اسم المريض	هند السيد مسعود
السن	
تاريخ اخذ العينة	
طالب التحليل	مركز صحة المرأة
عناية	مركز صحة المرأة
النوع	Female
العنوان	القاهرة
تاريخ استلام العينة	27/07/2025 14:47
تاريخ التقرير	29/07/2025 10:49

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor (PR) (IE2) Rabbit monoclonal primary antibody.
Sample recieved was treated by: - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX / ULTRA .
Sample recieved was treated by: - CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result :

ER: moderate nuclear staining of 30 % of tumour cells (core biopsy)
PR: NO nuclear staining of tumour cells (core biopsy)
HER2/neu protein over expression : strong continuous membrane staining of tumor cells (core biopsy).
Ki67 :positive nuclear staining of > 14 % of tumour cells (core biopsy)

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 5 / 8)
PR: NEGATIVE
HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3 +)
#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS > 14 % .

Pathologist:

د/تريزا ماهر

Lab Director:

د/تريزا ماهر



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يتم الاحتفاظ بالصل العينة لمدة 30 يوم من تاريخ استلامها



Ministry of health
Central health laboratories
Pathology department

Histopathology report

Report Number: 197

اسم المريض :	هند السيد مسعود	النوع :	Female
السن :	48 Y	العنوان :	القاهرة
تاريخ اخذ العينة :		تاريخ استلام العينة :	23/07/2025 10:20
طالب التحليل :	مركز صحة المرأة	تاريخ التقرير :	28/07/2025 10:52
عناية :	مركز صحة المرأة		

Primary sample :

Left breast core biopsy

comment :

Gross examination :

4 cores of greyish white tissue (T. I)

Microscopic examination :

Section examined revealed fibrocollagenous tissue infiltrated by malignant growth displaying the morphology of infiltrating duct carcinoma with medullary features .

Referral Lab :

Comment :

Diagnosis :

LEFT BREAST CORE BIOPSY: THE PICTURE IS COMPATIBLE WITH INFILTRATING DUCT CARCINOMA , WITH MEDULLARY FEATURES , GRADE II

Pathologist: Dr/Jehan Aziz

Lab Director: د/تريزا ماهر

يجب إحضار تقرير الصيغ
المناعية (ضروري)

(دعبداله محمدي)

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يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

NAME :Hind Sayed

Date : 21/7/2025

Bilateral breast sono-mammographic study

INDICATION :

Left palpable lump

REPORT :

- Scattered fibro glandular densities (ACR B)
- Bilateral scattered benign looking calcifications

Left breast :

- UOQ irregular loculated dense mass lesion
- Complementary US revealed UOQ irregular loculated hypoechoic lesion measuring 40x30mm distance 40 mm away from the nipple and 14mm from overlying skin
- The lesion shows no vascularity on colour Doppler
- Single indeterminate axillary lymph node is seen measuring 1.3x9mm with faint hilum

RIGHT BREAST :

- No speculations or grouped micro calcifications
- Complementary US revealed Complementary ultrasound revealed no solid or cystic mass lesion
- Normal skin /nipple /nipple areolar complex
- Non specific axillary lymph nodes

Conclusion

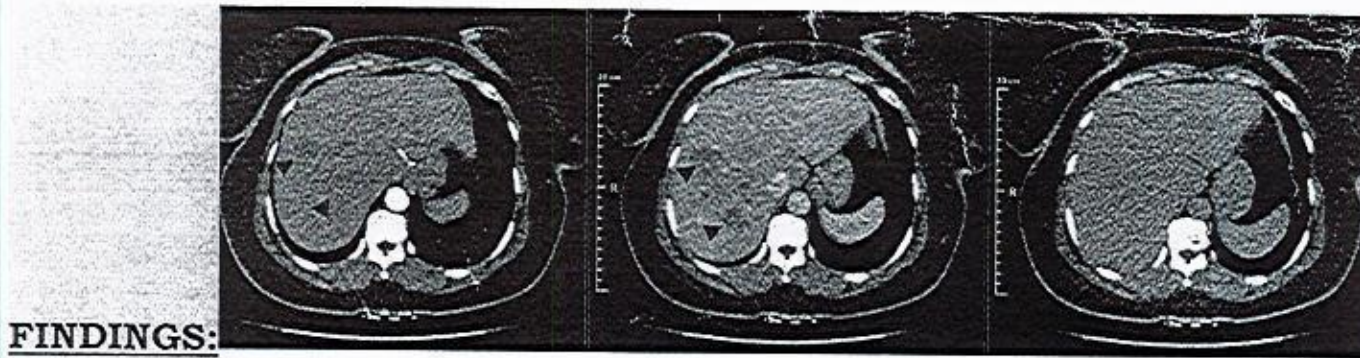
- ❖ Left breast suspicious mass lesion with ipsilateral indeterminate lymph node (BIRADS4) for Histopathological correlation
- ❖ Right breast : benign findings (BIRADS2)

DR.Dina Gaber ,FEBR

Patient Name: Hend Al Sayed Masoud Mohamed	Date : 01/09/2025
Exam : triphasic C.T Liver	Age : 48 Year(s)
Medical ID: 2397/2025	Pat. ID: 111139

MULTISLICE POST CONTRAST TRIPHASIC CT SCAN OF THE LIVER:

TECHNIQUE: Multislice axial CT images of the liver obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.



FINDINGS:

Pathologically proven left breast carcinoma .

Compared to the last study dated 26-8-2025 ,the current study revealed a stationary course of the previously noted:

- The liver is average in size showing homogenous CT density with multiple right hepatic segments 7 and 8 hypodense focal lesions ,reaching 32x32 mm in diameter with much smaller left hepatic segment 2 and 4a focal lesions , measuring 5-7 mm in diameter,with the later showing washout in the delayed phases.
- They show heterogeneous enhancement in the arterial and portal phases.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Cholecystectomy clips are noted.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Bulky uterus.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.

- . No ascites.
- D5 ,D9 and L3 vertebral small sclerotic lesions are noted.

OPINION:

- Clinically reported case of left breast carcinoma showing the above detailed changes with stationary course of the bilobar hepatic metastatic focal lesions.
- Few vertebral small sclerotic lesions, likely metastatic ,for bone scan correlation.

Dr.Sara Mahmoud, MD

Name : Hind El-Sayed Masoud Mohammed
Branch : Dokki

Date : 12-09-2025 11:08
Medical No. : 200616
Ins.Company : التأمين الصحي

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 48-year-old lady, with left breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen all over the surveyed skeleton.
- N.B: The prior CT reported vertebral lesions are not appreciated in our study.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

ECHO REPORT

NAME: هنادي ممدو DATE: 19/10/2025
ID NUMBER: ٩٢٩٧/٥٥ SEX: AGE:

EF	68%	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVEDS	2.3	2-4cm	LA	3.5	2-4cm
FS	29%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function Normal

SWMA No RWMA

LA and Ao Root Dimensions Normal

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.3

Intracardiac Masses and Effusion

DR. 